

Injury recovery — return to work

NOVA-EXO+ 3 cc gluteal IM + NOVA-ELITE 2 cc IM · physician treatment protocol

A standardized, physician-directed framework supporting recovery and return to work after tendon, muscle, and bone injuries — post-surgical or managed non-surgically. Given as two intramuscular injections alongside structured rehabilitation: an exosome dose in the gluteal muscle and a second dose in a large muscle near the injury.

Indications

- Tendon, muscle, or bone injury managed non-surgically
- Post-surgical repair of tendon, muscle, or bone — after surgical clearance
- Delayed recovery or prolonged time away from work or sport
- Persistent pain or weakness limiting return to full duty
- Adjunct to structured rehabilitation and work conditioning
- Plateaued progress with conservative care alone

Patient preparation

1. Verify indication; review operative report and imaging when available.
2. Obtain informed consent.
3. Record baseline VAS pain, function, and current work restrictions.
4. Post-surgical patients: confirm surgical clearance and wound healing first.
5. Position prone or side-lying; landmark the ventrogluteal site and the second muscle.
6. Prepare skin with alcohol or chlorhexidine.

POST-PROCEDURE CARE

- Observe 15 minutes following injection
- Soreness at the injection sites for 24–72 h
- Avoid NSAIDs and alcohol for 72 h when appropriate
- Rehab and work-conditioning continue as prescribed; no new loading of the injured structure without clearance

Contraindications

- Active local or systemic infection, including the surgical site
- Pregnancy or breastfeeding
- Unhealed surgical wound or compromised skin at an injection site
- Corticosteroid injection within 4 weeks
- Systemic corticosteroid use within 30 days
- Uncontrolled bleeding disorder or significant anticoagulation risk

Injection & distribution

TOTAL VOLUME · 2 INJECTIONS

5 cc

- | | | |
|---|--|-------------|
| 1 | NOVA-EXO+
Gluteal IM — ventrogluteal preferred | 3 cc |
| 2 | NOVA-ELITE
IM, large muscle near the injury — quadriceps, deltoid, or gastrocnemius per site | 2 cc |

Respect IM site limits — gluteal up to 3 cc per side, deltoid max 2 cc. Aspirate at every site; inject slowly. 22–25 G, length per site and habitus.

FOLLOW-UP & OUTCOME MEASURES

Week 2

Safety review

Week 6

Functional progress

Week 12

Primary outcome

VAS PAIN

ROM

STRENGTH

WORK STATUS

PT MILESTONES

SATISFACTION

ADVERSE EVENTS

REGULATORY

Intended for physician-directed clinical and investigational use. Providers are responsible for compliance with applicable federal, state, facility, IRB, and professional practice requirements.

Today you received two intramuscular injections of exosome products — one in the muscle of your hip and one in a large muscle near your injury. The products used are made from donated birth tissue and are not FDA-approved treatments. This page tells you what is normal in the days ahead, how to care for yourself, and when to call us.

DATE OF TREATMENT

SITE(S) TREATED

NEXT VISIT

WHAT TO EXPECT

Changes are gradual — do not judge the result in the first days

FIRST 72 HOURS

Soreness at both injection sites — much like after a vaccine — is common and usually settles in one to three days.

WEEKS 1–4

Continue physical therapy and rehab exactly as prescribed — this treatment adds to your rehab plan, it does not replace it.

WEEKS 4–12

Progress is measured at follow-up against your baseline pain, strength, and work status.

DO

- Keep every physical therapy and rehab appointment
- Follow your current work restrictions until your physician updates them
- Use cold packs 10–15 minutes at a time for comfort
- Take acetaminophen (Tylenol) for soreness if needed

AVOID

- Anti-inflammatory pain relievers for 72 hours
- Alcohol for 72 hours
- Loading the injured area beyond your current restrictions
- Massage of the injection sites for 48 hours

CALL US RIGHT AWAY IF

For an emergency, call 911 first

- An injection site becomes increasingly hot, red, or swollen
- Calf pain or swelling, or shortness of breath
- Fever above 101 °F or shaking chills
- Severe pain not controlled by acetaminophen and cold packs

CLINIC PHONE

AFTER-HOURS PHONE

NOTES FROM YOUR VISIT